

Dashboard Simluation Report

Scenario Used	What the Dashboard	Alerts Triggered	Recommended Actions	Weaknesses Identified	Response to Each Weakness
Patient discharged 4 days ago. No outreach contact documented. Scheduled outpatient appointment missed yesterday with no attendance record. Assigned case manager carrying 34 active post-discharge patients including multiple elevated-risk cases.	Critical Alerts Panel activated first, requiring acknowledgment before caseload view loaded. Patient flagged at top of panel with three simultaneous threshold breaches: outreach window breached by 72 hours beyond the 48-hour elevated-risk requirement; missed appointment without same-day review breached by more than 24 hours; documentation completeness at 0% across all active workflow stages. Continuity Status Indicator displayed CRITICAL with documentation completeness modifier. Caseload concentration flag surfaced 34 active patients with multiple overdue contacts visible to CM Lead.	Outreach window breach alert (Metric 2): elevated-risk patient, no contact documented, 72 hours beyond threshold. Missed appointment alert (Metric 4): no same-day review, no DP6 triage decision, breached by more than 24 hours. Documentation completeness alert (Metric 8): zero records across outreach attempts, structured outreach initiation, escalation notifications, and DP6 triage decision. Ownership and risk tier verification flags: risk tier auto-populated and unconfirmed. Caseload concentration alert (Metric 7): 34 active patients, overdue contacts flagged to CM Lead. Automatic CM Lead notification fired on dashboard open for all active breaches. Treating consultant notification flagged as required and overdue.	Step 1: Reconstruct and document any coordination actions taken but not recorded before logging new activity. Step 2: Enter DP6 triage decision (logistics barrier or clinical concern) — required before resolution pathway activates. Step 3: Initiate immediate outreach across all available channels and log each attempt in real time. Step 4: Formally document structured outreach protocol initiation as a required separate field. Step 5: Confirm CM Lead and treating consultant notification with timestamp. Dashboard locked DP6 resolution pathway until triage decision field was completed. Crisis intervention pathway prompted automatically if clinical concern selected at DP6. CM Lead received automatic notification including breach durations, documentation completeness score, and caseload load flag without case manager initiation.	Dashboard only generates alerts when opened; background threshold monitoring not confirmed as architecture, creating a gap during the 4-day window when no alerts reached the case manager or CM Lead in real time. Zero documentation treated as zero coordination, which may misrepresent genuine but unrecorded outreach activity and create a disciplinary framing rather than a supportive one. Risk tier classification inherited from discharge assessment without independent verification, meaning incorrect classification produces wrong outreach thresholds with no visible flag. Dashboard displays reassuring statuses (Stable, Monitoring) based on workflow position, not clinical condition, leaving silent deterioration between contact points undetectable. High caseload volume with multiple simultaneous critical alerts creates alert saturation, diluting prioritization and reintroducing the manual triage burden the dashboard was designed to reduce. Informal ownership transfers not captured by mismatch detection logic, leaving patients monitored under an original assignee who no longer holds operational responsibility.	Dashboard absence during 4-day window: Fixed in prototype — threshold monitoring specified as a background process independent of login events, with CM Lead notifications firing on elapsed time rather than dashboard sessions. Undocumented action misread as absent action: Acknowledged as limitation — retrospective documentation prompt included as partial mitigation; CM Lead notification language revised to distinguish documentation absence from coordination absence and require a supervisory conversation before conclusions are drawn. Risk tier accuracy: Acknowledged as limitation — independent verification is outside prototype scope; partial mitigation added by prompting case manager to reconfirm risk tier at first outreach contact. Silent clinical deterioration between contacts: Determined out of scope — dashboard is a coordination support tool, not a clinical assessment instrument; prototype documentation states explicitly that Stable and Monitoring statuses reflect workflow position only, not clinical safety. Alert saturation under high caseload: Fixed in prototype — Critical Alerts Panel capped at three visible alerts per case manager; alerts exceeding actionable capacity automatically redistributed to CM Lead as an overflow category. Informal ownership transfer invisibility: Fixed in prototype — ownership reconfirmation prompt added for any case with no logged case manager action within 48 hours, replacing assumption of continuity with active periodic confirmation.